

REFERENCES

- BEN-PORATH, Y. S., & BUTCHER, J. N. (1989). The psychometric stability of rewritten MMPI items. *Journal of Personality Assessment*, 53, 645-653.
- BUTCHER, J. N. (1989). *Minnesota Multiphasic Personality Inventory-2, user's guide, the Minnesota Report: Personnel Selection System*. Minneapolis: University of Minnesota Press.
- BUTCHER, J. N., DAHLSTROM, W. G., GRAHAM, J. R., TELLEGEN, A., & KAEMMER, B. (1989). *Manual for administration and scoring, MMPI-2, Minnesota Multiphasic Personality Inventory-2*. Minneapolis: University of Minnesota Press.
- BUTCHER, J. N., GRAHAM, J. R., WILLIAMS, C. L., & BEN-PORATH, Y. S. (1990). *Development and use of the MMPI-2 Content Scales*. Minneapolis: University of Minnesota Press.
- COLLIGAN, R., & OFFORD, K. P. (1988). Contemporary norms for the Wiggins Content Scales: A 45-year update. *Journal of Clinical Psychology*, 44, 23-28.
- KOHUTEK, K. (1992). Wiggins Content Scales and the MMPI-2. *Journal of Clinical Psychology*, 48, 215-218.
- LEVITT, E. E. (1990). A structural analysis of the impact of MMPI-2 on MMPI-1. *Journal of Personality Assessment*, 55(3 & 4), 562-577.
- WIGGINS, J. S. (1966). Substantive dimensions of self-report in the MMPI item pool. *Psychological Monographs*, 80(22, Whole No. 630).

NAIVETE AND NEED FOR AFFECTION AMONG PEDOPHILES

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MMPI scores for items on the Harris and Lingoes (1955, 1968) subscales Hy2 (need for affection) and Pa3 (naivete) were compared among pedophiles ($n = 50$), non-sexually deviant psychiatric patients ($n = 25$), and a general control group ($n = 50$). Results indicated that pedophiles did not demonstrate a "naive need for affection," but, rather, a cynical need for affection. Scores on the Pedophilia (Pe) Scale (Toober, Bartelme, & Jones, 1959) also were compared, and no significant differences were found between the pedophiles and either control group, although a difference was found between the scores of the psychiatric control group and those of the general sample. Selected items from scale 4 (Psychopathic Deviate) and scale 9 (Hypomania) also produced no significant differences among groups.

Recent research seeks to identify psychological characteristics of pedophiles, i.e., individuals who are defined by the DSM-III-R (American Psychiatric Association, 1987) as suffering from significant feelings of sexual attraction toward prepubertal children. Pedophilic characteristics have been investigated through psychophysiological (Abel, Blanchard, & Barlow, 1981), questionnaire (S. Johnston & Anderson, 1985), and psychometric (McCreary, 1975; Panton, 1978, 1979) instruments as well as through analysis of drawings (F. Johnston & Johnston, 1986). Because evaluations of an individual's propensity to sexual deviance often occur in a forensic context, actuarial

instruments with subtle items based on issues of social awareness and interpersonal style can have utility (S. Johnston & Anderson, 1985).

The impetus for the present study was the observation that many individuals referred for evaluation under a Penal Code Section that pertains to sexual child abuse had elevations on the Harris-Lingoes Hy2 ("need for affection") and PA3 ("naivete") subscales of the MMPI. These elevations indicate eccentricities in the subject's view of the interpersonal world that fit with our clinical impression gained from the treatment of pedophiles. These pedophiles often verbalize in clinical interviews and during treatment sessions a longing for simple non- or pre-ambivalent relationships; that is, a world in which "roses have no thorns."

Although the MMPI is the most widely used personality inventory (Anastasi, 1988), its utilization in predicting pedophilic behavior is sparse and at times equivocal and contradictory. Toobert et al. (1959), who used an incarcerated population, developed a Pedophilia (Pe) scale; other authors have found that the MMPI discriminates between violent and nonviolent sexual offenders (Panton, 1978). Levitt (1989), however, recently has questioned the specificity of the Pedophilia scale and has noted that it appears to identify common characteristics of several groups of sexual deviates, not just pedophiles.

Therefore, we studied the Pedophilia scale as well as the Harris-Lingoes subscales Hy2 (need for affection) and PA3 (naivete). Additionally, content-obvious items were selected from the scale 4 (Pd) and scale 9 (Ma) scales in order to measure social alienation, sexual acting-out, and impulsivity. The hypothesis was that these five groups of items (i.e., from the Pedophilia Scale, Hy2, PA3, Psychopathic Deviate, and Hypomania) could distinguish reliably between a sample group of pedophiles and non-sexually deviant groups. The complete list of items is shown in Table 1.

METHOD

Subjects

The subjects of this study were 125 adult males, who ranged in age from 19 to 80 years, from a large California county. Participants were obtained from all racial groups with varying amounts of education, from "some high school" to "college degree completed," and research subjects were selected on the basis of their membership in one of the three relevant research design groups.

The first group consisted of 50 subjects who either entered pleas of "guilty" or "no contest" to criminal charges that alleged child sexual abuse. All cases were derived from the files of a forensic psychiatrist. As a part of their court-ordered examinations to determine whether they were suitable for probation and amenable to treatment, these subjects were administered the MMPI in its complete form. Although these data are currently public information because the results of the psychiatric examination were reported in open court, the participants remain anonymous to ensure confidentiality.

The second group consisted of 25 subjects randomly selected from the files of the same forensic psychiatrist with the exception that these subjects were neither involved in, nor did they have, a history of child molestation. These clients had been referred for a variety of reasons including personal injury suits, disability claims, and criminal cases of a nonsexual nature. Each of these clients also had been administered the MMPI in its complete form.

The third group consisted of 50 control subjects from the same geographical area who volunteered to complete a questionnaire composed of the above-indicated items from the MMPI. Research subjects within this general control group were obtained by canvassing adult males at a local shopping center. Approximately one-third of the adult male subjects solicited at this shopping center consented to be administered the research questions. All subjects were assured of the anonymity of their responses.

Table 1
MMPI Items Included in the Research Measure

Hy2 (false)

- 26. I feel it is certainly best to keep my mouth shut when I am in trouble.
- 71. I think a great many people exaggerate their misfortunes to gain the sympathy and help of others.
- 89. It takes a lot of argument to convince most people of the truth.
- *93. I think most people would lie to get ahead.
- *109. Some people are so bossy that I feel like doing the opposite of what they request, even though I know they are right.
- *124. Most people will use somewhat unfair means to gain profit or an advantage rather than to lose it.
- 136. I commonly wonder what hidden reasons another person may have for doing something nice.
- 162. I resent having anyone taking me in so cleverly that I have had to admit that it was one on me.
- 234. I get mad easily and then get over it soon.
- 265. It is safer to trust nobody.
- 289. I am always disgusted with the law when a criminal is freed through the arguments of a smart lawyer.

PA3 (true)

- 314. Once in a while I think of things too bad to talk about.

PA (false)

- *93. I think most people would lie to get ahead.
- *109. Some people are so bossy that I feel like doing the opposite of what they request, even though I know they are right.
- 117. Most people are honest chiefly through fear of being caught.
- *124. Most people will use somewhat unfair means to gain profit or an advantage rather than to lose it.
- 313. The man who leaves valuable property unprotected is about as much to blame for its theft as the one who steals it.
- 316. I think nearly anyone would tell a lie to keep out of trouble.
- 319. Most people inwardly dislike putting themselves out to help others.
- 348. I tend to be on my guard with people who are somewhat more friendly than I had expected.

PE (true)

- *16. I am sure I get a raw deal from life.
- 53. A minister can cure disease by praying and putting his hand on your head.
- *67. I wish I could be as happy as others seem to be.
- 95. I go to church almost every week.
- 179. I am worried about sex matters.
- 202. I believe I am a condemned person.
- 206. I am very religious (more than most people).
- 219. I would like the work of a building contractor.
- 248. Sometimes without any reason or when things are going wrong, I feel excitedly happy, "on top of world."
- 260. I was a slow learner in school.
- 332. Sometimes my voice leaves me or changes, even though I have no cold.
- 390. I have often felt badly over being misunderstood when trying to keep someone from making a mistake.

458. The man who had most to do with me when I was a child (such as my father/step-father) was very strict.

490. I read in the Bible several times a week.

PE (false)

57. I am a good mixer.

76. Most of the time I feel blue.

133. I have never indulged in any unusual sex practices.

276. I enjoy children.

435. I usually would prefer to work with women.

556. I am very careful about my manner of dress.

PD (true)

*16. I am sure I get a raw deal from life.

24. No one seems to understand me.

*67. I wish I could be as happy as others seem to be.

84. These days I find it hard not to give up hope of amounting to something.

102. My hardest battles are with myself.

106. Much of the time I feel as if I have done something wrong or evil.

215. I have used alcohol excessively.

PD (false)

20. My sex life is satisfactory.

37. I have never been in trouble because of my sex behavior.

294. I have never been in trouble with the law.

MA (true)

22. At times I have fits of laughing and crying that I cannot control.

97. At times I have a strong urge to do something harmful or shocking.

156. I have had periods in which I carried out activities without knowing later what I had done.

Note.—Items with an asterisk(*) are on more than one scale.

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Research Measure

Both pedophiles and psychiatric patients were given the standard version of the MMPI. The general control group was administered the 49 MMPI items listed in Table 1 (i.e., those items drawn from the Hy2, PA3, PD, MA, and PE scales).

Procedure

Scores were obtained on the 49 MMPI items for the three research groups (i.e., the pedophiles, the psychiatric patients group, and the general control group). Standardized instructions on the test booklets directed each participant to answer each statement either "true" or "false" as it applied to him. No items were to be left unanswered, and frequent checking between test question numbers and answer sheet numbers was encouraged to ensure that replies corresponded to the appropriate questions.

Statistical Analysis Plan

Test scores for the three groups were computed and subjected to an analysis of variance to identify any significant differences. Only those scales that were significant

at the .05 level of probability were retained for further analysis by post hoc comparison, using Tukey's (1949) test of honestly significant differences in order to determine which groups differed significantly on each MMPI subscale.

Research subjects' scores on all of the research measures were obtained by summing the subjects' number of responses demonstrative of pathology on each of the procedures. The scores from all of the research measures were subjected to an analysis of variance (ANOVA) in order to determine whether significant differences existed among the relevant research groups. Analyses were carried out using the Statistical Package for the Social Sciences Version X (Norusis, 1988).

RESULTS

Only those scales that contained significantly different scores were subjected to further statistical analysis. Included in this group were the Pedophilia (PE) scale, $F(2,121) = 4.11, p < .003$, the need for affection (HY2) subscale, $F(2,121) = 6.18, p < .0028$, and the naive (PA3) subscale, $F(2,121) = 6.23, p < .003$. The Psychopathic Deviate (PD), $F(2,121) = .811, p < .4467$, and Mania (MA), $F(2,121) = 1.14, p < .3233$, items produced nonsignificant results and, therefore, were dropped from further study. Table 2 gives the means and standard deviations for each of the remaining groups.

Table 2
Raw Score Means and Standard Deviations

	<i>M</i>	<i>SD</i>
HY2 (need for affection)		
Pedophiles	7.88	2.69
Patients	6.36	3.05
General	7.80	3.72
PA3 (naivete)		
Pedophiles	5.12	2.17
Patients	4.40	2.33
General	5.93	2.57
PE (pedophilia)		
Pedophiles	7.09	1.67
Patients	8.72	2.81
General	5.20	3.57

As a point of comparison, it also should be noted that none of the overall means on any of the variables (HY2, PA3, and PE) in the present study was more than one standard deviation away from the mean scores of the original samples reported by Harris and Lingoes (1955) and Toobert et al. (1959).

The HY2, PA3, and PE scales then were subjected to a post hoc test to determine which group(s) differed significantly. Tukey's test for honestly significant differences (HSD) was selected with ranges set at the .05 significance level. Table 3 gives the results of this procedure.

The mean scores obtained on the PE scale were 7.09, 8.72, and 5.20 for the pedophiles, the psychiatric patients, and the general sample, respectively. The post hoc results indicated that the psychiatric group's score on the PE scale was significantly higher than that of the general sample, but there was no significant difference between the scores

Table 3
Comparison of Group Means – Tukey's HSD

	Group	Pedo.	Patient	General
PE				
<i>M</i>				
7.09	Pedo.			
8.72	Patient			
5.20	General		***	
Variable Hy2				
<i>M</i>				
7.88	Pedo.		***	
6.36	Patient			
7.80	General	***		
Variable PA3				
<i>M</i>				
5.12	Pedo.			
4.40	Patient			
5.93	General	***	***	

*** ($p < .05$).

of the psychiatric patients and the pedophiles or between the scores of the pedophiles and the general sample.

The mean scores on the Hy2 subscales were 7.88, 6.36, and 7.84 for the pedophiles, the psychiatric patient group, and the general sample, respectively. The post hoc results indicated that the pedophiles and the general sample did not differ significantly from each other in their scores, but that both groups obtained significantly higher scores on this subscale than did the psychiatric patient group.

The mean scores obtained by pedophiles on the PA3 subscale were 5.12, 4.40, and 5.93 for the pedophiles, the psychiatric patients, and the general sample, respectively. This test demonstrated significant differences between two pairs: The general sample obtained a higher score than the psychiatric patients, and the general sample obtained a lower score than the pedophiles.

DISCUSSION

The statistical analyses performed on the present research measure revealed a number of interesting results. Toobert et al.'s Pedophilia Scale failed to distinguish between pedophiles and the general sample. Rather, virtually all of the significant differences observed were between the psychiatric patients and the general sample (i.e., subjects who were recruited at shopping centers to participate in the study). Thus, pedophiles were indistinguishable from the psychiatric patients on this measure. This may reflect a substantial selection bias in that the subjects in Toobert et al.'s study were institutionalized in a state hospital for the criminally insane, whereas the pedophiles in the present study participated in the psychiatric examination specifically because of their eligibility for the grant of probation and outpatient treatment. Thus, even though we do not have follow-up data on the pedophiles in the present study, it is a reasonable inference that a substantial proportion of these individuals were granted a term of probation rather than being hospitalized in state hospitals or prisons. Therefore, Toobert et al.'s group may have been more generally impaired than those studied here.

With regard to the differences in scores of the three research groups on the Hy2 or "need for affection" items, the scores of the psychiatric patients differed significantly from the scores obtained by pedophiles and normal controls. These data indicate that pedophilic males may not differ from other men with regard to their need for affection (at least as measured by the Hy2 items).

Although inconsistent with the original hypothesis, the data showed that the general sample generated higher scores than either the pedophiles or the psychiatric patients on the PA3 scale items. This original hypothesis was based on the idea that pedophiles in comparison with controls would reveal more naivete. If one examines more closely items from the PA3 scale, however, the idea that it is measuring primarily naivete must be questioned. Caldwell (1988) has suggested, for example, that this subscale be called the "moral righteousness" scale. He points out that high scores indicate a trusting point of view, while "low score responses read as if straight from Machiavelli . . . of course people act from their own self-interests, and they will go to great lengths to protect their advantages" (p. 27)—a point of view that we find to be entirely consistent with the exploitive stance central to pedophilic behavior. It seems likely, then, that the original hypothesis that pedophiles would score higher than the controls on PA3 was in error because of lack of experience in comparing PA3 scores of what might be called "normal persons" (rarely seen in psychiatric practice) with pedophiles and other types of disturbed individuals. Thus, upon a closer examination of the PA3 items (with hindsight of the present statistical analysis), it does not seem surprising that pedophiles would demonstrate greater self-righteousness and that the general sample would demonstrate a greater capacity for trust.

The results of the present research offer favorable implications for the hypothesis that pedophiles can be differentiated in terms of their naivete and need for affection. However, these results suggest that pedophiles may not be "naively needy" as hypothesized, but, instead, may be cynically and/or moralistically needy. This finding also is consistent with the original observation that pedophiles in clinical interviews or therapy sessions express a longing for non- or pre-ambivalent relationships. Such longings may have been seen as deriving not from the blind trust implied by naivete, but from the distrust associated with exploitive relationships.

In conclusion, caution must, of course, be exercised in interpreting these results given the forensic setting in which these data were obtained and the probable attempt on the part of the court-referred pedophiles to present as favorable a self-picture as possible. However, the results of the present research indicate that subtle or non-obvious items can distinguish reliably among pedophiles and other groups.

REFERENCES

- ABEL, G. L., BLANCHARD, E. B., & BARLOW, D. H. (1981). Measurement of sexual arousal in several paraphilias: The effects of stimulus modality, instructional set and stimulus content on the objective. *Journal of Behavior Research and Therapy*, 19, 25-33.
- AMERICAN PSYCHIATRIC ASSOCIATION. (1987). *Diagnostic and statistical manual of mental disorders (3rd ed. rev.)*. Washington: Author.
- ANASTASI, A. (1988). *Psychological testing* (6th ed.). New York: Macmillan.
- CALDWELL, A. B. (1988). *MMPI Supplemental Scale manual*. Los Angeles: Caldwell Report.
- HARRIS, R., & LINGOES, J. (1955). *Subscales for the Minnesota Multiphasic Personality Inventory: An aid to profile interpretation*. University of California, San Francisco, Department of Psychiatry.
- HARRIS, R., & LINGOES, J. (1968). *Subscales for the Minnesota Multiphasic Personality Inventory*. Mimeographed materials, Langley Porter Clinic.
- JOHNSTON, F. A., & JOHNSTON, S. A. (1986). Differences between human figure drawings of child molesters and control groups. *Journal of Clinical Psychology*, 42, 638-647.
- JOHNSTON, S. A., & ANDERSON, R. E. (1985). Development of scales to measure sexual aggressives. *International Journal of Offender Therapy and Comparative Criminology*, 3, 122-134.

- LEVITT, E. (1989). *The clinical application of MMPI special scales*. Hillsdale, NJ: Erlbaum.
- MCCREARY, C. P. (1975). Personality differences among child molesters. *Journal of Personality Assessment*, 39, 591-593.
- NORUSIS, M. J. (1988). *SPSS-X introductory statistics guide for release 3*. Chicago: SPSS.
- PANTON, J. H. (1978). Personality differences appearing between rapists of adults, rapists of children, and non-violent sexual molesters of female children. *Research Communications in Psychology, Psychiatry, and Behavior*, 3, 385-392.
- PANTON, J. H. (1979). MMPI profile configurations associated with incestuous and non-incestuous child molesters. *Psychological Reports*, 45, 335-338.
- TOOBERT, S., BARTELME, K. F., & JONES, E. S. (1959). Some factors related to pedophilia. *Journal of Social Psychiatry*, 4, 272-279.
- TUKEY, J. W. (1949). One degree of freedom for non-additivity. *Biometrics*, 5, 232-242.

CLINICAL CONSTRUCT VALIDITY OF THE HOLDEN PSYCHOLOGICAL SCREENING INVENTORY (HPSI)

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With a sample of 64 adult psychiatric patients, we examined the construct validity of the Holden Psychological Screening Inventory (HPSI) for a clinical population. Factor analysis supported the three-dimensional nature of the HPSI and the appropriateness of the instrument's scoring key. Reliability information confirmed the internal consistency of the inventory's scales. Clinical staff ratings were used as criteria, and analyses indicated that HPSI scales demonstrated convergent validity. Overall, findings extend previous nonclinical research on the HPSI and suggest that the instrument possesses construct validity for clinical applications.

The Holden Psychological Screening Inventory (HPSI; Holden, 1991) is a 36-item, 5- to 7-minute, self-report questionnaire designed to identify individuals who might benefit from more elaborate individualized assessment of psychological dysfunctioning. This study investigated the construct validity of the HPSI with a sample drawn from a clinical population.

The HPSI was constructed through multivariate methods to be a screening instrument that: (a) is psychometrically sound; (b) is quick and easy to administer; (c) assesses

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